

إسم المريض ربيعاً : نهال سامى عبدالله عثمان
المحافظة : القاهرة الحي / المدينة : دار السلام
رقم البطاقة : 28311100104483 رقم التليفون : 01143683840
العمر(تاريخ الميلاد) : 10-11-1983 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

GR 1-4
ر. د. يوسف
أستاذ
الدراسات
الاسلامية
جامعة
البحرين
صحة البيانات المرفقة
الاجتهاد

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

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الوظيفة: ----- التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نَفَقَات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

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Name: نهال سامى عبدالله عثمان National ID: 28311100104483	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



21/10/2025 G

<u>Name</u>	Cheng L. L.	<u>MRN</u>	025100
<u>Age</u>	45	<u>ID</u>	
<u>Menopausal status</u>	pre	<u>Phone</u>	011 5076 333
<u>NRS SCORE</u>		<u>Marital status</u>	
<u>BMI</u>		<u>Family history</u>	
<u>CO morbidity</u>		<u>REASON OF MDT</u>	

Clinical Data

BX: JOC CII			
ER: 8/8	PR: 8/8	HER 2: 0	KI67: 46/

R

L

Sonomammography:

10/19/20 11 breast lolly adored
11 YIP more (inhibiting sk)

Mets Workup:

3/10/2021 at GNP → Fr Leptoz
lake sol n neds
① multiple osteolytic bag l



TNM

Recommendations:

Interventions:
Denon MBC to Boes

giving history of pathological fracture of 11th femur.

gr didn't attend Δ 0 file 1 HSE

For MDT to be on ~~the~~ ^{the} ~~list~~ ^{list} of pathogens

MDT OUTCOME & PLAN

Dr. Tansal & Co. R. Shyl to Arden R.
tobacco P. Zoladex
- 2 - 4. M. 19. 6. 8. 4. 3.
SIGNATURE

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatry
					

Patient Name	نهال سامی عبداللہ عثمان	Age	41
Sex	f	Special habits	
Family History		Comorbidity	
Surgical History		Telephone	01119076333
Menopausal Status	pre	Marital Status	
ID	2804/2025	Medical ID	28311100104483
Date	26-10-2025	Reason for MDT	

Medical Report

Affected breast: LEFT

Pathology: IDC GII

ER 8/8 PR 8/8 HER2 0 KI67 :40%

T N M:

S.M:

10/9/2025 : locally advanced LIQ breast mass BIRADS 5

Metsworkup:

8/*10/2025 : ct cap: multiple osteolytic destructive bony lesions with extraosseous soft tissue component for assessment by bone scan

Doctor's notes:

Gave a history of pathological fracture of femur

Recommendations:

Pt didnt attend: 1- report from orthopedics requested, 2- films of MRI

For MDT to decide on

1- fixation of pathological fracture

2- start ?????

MDT DECISION

For:tamoxifen and zoladex first 3 months then shift to aramadex and zoladex
Zometa monthly for first 6 months then once every 3 months

Name	د/حنان عزت	د/رائیا شفیق
Signature		

Patient Name : الأستاذ / نهال سامي عبدالله عثمان
Patient No. : 28311100104483
Path No. : 2895/25
Sex : Female
Age : 41 Y
Referring Doc :

Request No. : 13001064
Request Date : 23/09/2025
Verification Date : 27/09/2025
Financial Category : جوماتاف روسي - مياثره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast UIQ mass BIRADs 5.
Nature of Specimen: Tru-cut biopsy, collected at 21/09/2025

Gross Examination:

- Four grayish white tissue cores up to 1.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (2+2+2) Grade II.
- * Ductal carcinoma in situ: Minimal (one duct) high grade (cribriform with comedo necrosis)
- * Perineural invasion: No
- * Angioinvasion: No
- * Microcalcifications: No
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).
 - Her2: No membrane reaction in tumor cells (score 0).
 - Ki67: 40% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with minimal high grade DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).

Verified by :

Printing Date : 21/10/2025 13:01:16

Page 1 of 2

Printing By : hermal



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Nehal Sami Abd Allah Othman	Date : 10/09/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 41 Year(s)
Medical ID: 10578	Pat. ID: 111398

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Left breast lump.

- **Breast density:** Scattered densities of glandular tissue....Matching ACR B.
- Left UIQ irregular shaped high density mass .
- No detected definite suspicious mass density, architectural distortion or pathological micro-calcification in the right breast.

By complementary breast ultrasound:

- Heterogeneous predominantly glandular echo-texture of both breasts.

Left Breast:

- An irregular shaped infiltrative mass is seen at 11 o'clock ,zone 3 ,that measures (5 x 4.5 cm) infiltrating the overlying skin .
- Normal nipple .
- Mass lesion is seen in the left axillary region mostly nodal ,it measures (3.2 x 1.5 cm) .

Right Breast:

- Normal skin& nipple .
- No detected definite suspicious solid or cystic masses in the left breast.
- No detected suspicious right axillary lymph nodes .

CONCLUSION:

- Left breast :locally advanced left UIQ mass (BI-RADS 5 category.
- Normal right breast (BIRADS 1 category).

Much Obliged

DR.MONA ZAKY.MD

professor-Breast imaging-Mansoura university

Signature Dr. Mona Zaki

Patient Name: Nehal Sami Abd Allah Othman	Date : 08/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 42 Year(s)
Medical ID: 2804/2025	Pat. ID: 111398

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND

PELVIS:

Findings:

I Chest:

- The left breast parenchyma shows an ill-defined necrotic soft tissue mass measures about 5.2 x 3.8 cm in axial dimensions. It is surrounded by smaller satellite lesions.
- Left pulmonary baal atelectatic bands.
- Otherwise clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with fatty infiltration of its parenchyma. It shows a hypodense non-enhancing focal lesion at segment VII measures about 2 cm in diameter.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** multiple osteolytic bony lesions are seen scattered at the vertebrae, pelvic bones and left proximal femur with destructive changes seen at the right iliac bone and left proximal femoral shaft and greater tuberosity associated with extraosseous soft tissue components.

The extraosseous component at the right iliac bone measuring about 2 cm and seen extending to the right neural foramen.

Opinion:



Patient Name: نهال سامي عبد الله

Study Date: 12/09/2025 10:26:37 AM

M.S.C.T AND M.R.I. OF BOTH HIP JOINTS

Technique:

- Multiple pulse sequences in different planes before & after IV contrast. MSCT films.

Findings: Given history of breast neoplasm.

- The hallmark of the examination is the marrow infiltrative lesion seen implicating the left proximal femur predominantly involving its neck and intra-trochanteric region with subsequent pathological fracture displaying osteolysis, multifocal cortical interruption and subsequent small extra-osseous soft tissue component "mainly superiorly and laterally abutting nearby muscles" along with avulsed few bony fragments especially originating from the greater trochanter. Such lesion displays iso to low T1, high T2 and shows avid fairly homogenous post-contrast enhancement. A picture of aggressive marrow infiltrative lesion at left proximal femur (most likely neoplastic and probably metastatic bony deposit) causing osteolysis and pathological fracture as described.
- Another smaller enhancing marrow infiltrative lesion at postero-medial aspect of right iliac bone with cortical interruption and small extra-osseous component medially.
- Additional global marrow heterogeneity of the hip and pelvic bones with scattered tiny marrow infiltrative foci including the left femoral head.
- Mild intra-articular synovial effusion, more on the left side.
- The hip joint spaces are preserved. No femoral heads collapse.



YOURS SINCERELY,

DR. RAMY EL-TOUKHY, MD

LECTURER OF RADIO-DIAGNOSIS
CAIRO UNIVERSITY

Name : Nihal Sami Abdullah Othman
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 21-08-2026 08:46
Medical No. : 444325

Tc^{99m} MDP FOLLOW UP SKELETAL SCINTIGRAPHY

History

43 year-old female patient, with history of treated metastatic left cancer breast to the bone. She gave history of left hip replacement, referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

Considering the previous bone scan report dated 10/07/2024, the current study revealed regression of the metastatic bone disease evidenced by:

- Near complete disappearance of the previously depicted lesions at upper cervical spine, LV1, LV4, LV5, right 4th costo-vertebral junction and right iliac bone.
- Marked regression in term of size and tracer uptake intensity is also noted at the other lesions involving the right sacro-iliac region and lower shaft of the left femur.
- No newly developed lesions could be elicited on the current study.
- Degenerative changes were also noted at different levels of the spine and around big joints.
- Left hip prosthesis shows active tracer uptake around its upper part, likely osteoarthritic changes.

OPINION

- Remarkable regression of the metastatic bone disease with decrease total number of the lesions and no newly developed ones as described above indicating good response to the given therapy.

Much Obligated
Dr. Alaa Nagy, MD
Nuclear medicine